



Submitted via Regulations.gov
Re: HHS Health Sector AI RFI

February 23, 2026

Assistant Secretary for Technology Policy
Office of the National Coordinator for Health Information Technology
Department of Health and Human Services
Washington, DC

Dear Sir or Madam,

Integrating the Healthcare Enterprise (IHE) appreciates the opportunity to respond to the *Request for Information: **Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care.***

At the outset, IHE believes it is important to state clearly that the Assistant Secretary for Technology Policy / Office of the National Coordinator for Health Information Technology (ASTP/ONC) has been a principal sponsor, strategic collaborator, and long-standing policy partner of IHE for more than two decades. ASTP/ONC's sustained engagement has helped translate federal health IT priorities into practical, testable, real-world interoperability across the U.S. healthcare system. This collaboration has supported national objectives related to patient safety, data exchange, security, clinical workflow integration, and public trust; foundational elements that are equally critical to the successful adoption of Artificial Intelligence ("AI") in clinical care.

IHE is a global, non-profit, multi-stakeholder initiative that convenes clinicians, vendors, health systems, standards development organizations, and regulators to develop implementation guidance ("IHE Profiles") and to validate those specifications through rigorous, public, real-world testing events (Connectathons). IHE does not develop AI algorithms, sell products, certify vendors, or provide clinical services. Our role is to ensure that complex health IT ecosystems, including AI-enabled systems, can reliably exchange data, integrate into clinical workflows, and be governed safely at scale.

Interoperability as a Prerequisite for Clinical AI Adoption

Across healthcare, and particularly in data-intensive domains such as medical imaging, IHE's experience consistently demonstrates that the most significant barriers to AI

adoption are not algorithmic performance, but fragmentation of data, workflows, and accountability across systems.

Effective clinical AI depends on:

- Timely access to high-quality clinical data from multiple systems
- Reliable clinical context (orders, protocols, patient characteristics)
- Predictable integration into operational workflows
- Traceability of AI outputs for audit, oversight, and continuous improvement

Absent robust interoperability, AI tools become difficult to deploy safely, expensive to maintain, and challenging to evaluate once in use. These challenges increase operational risk for providers, slow innovation, and ultimately undermine public trust.

IHE's ongoing work reflects this reality. For example, IHE Radiology has developed the **AI Results (AIR)** and **AI Result Assessment for Imaging (AIRA)** Profiles, which define how AI-generated findings and their subsequent assessment can be encoded, stored, queried, and retrieved using established healthcare standards such as DICOM and HL7, while aligning with modern web-based interoperability approaches including DICOMweb and HL7 FHIR. Rather than introducing parallel AI-specific data models, this work demonstrates how existing standards infrastructure can be profiled and used in combination to support interoperable, traceable AI integration within clinical systems.

Links to IHE Radiology AI specifications:

AI Results (AIR)

https://www.ihe.net/uploadedFiles/Documents/Radiology/IHE_RAD_Suppl_AIR.pdf

AI Result Assessment for Imaging (AIRA)

https://www.ihe.net/uploadedFiles/Documents/Radiology/IHE_RAD_Suppl_AIRA.pdf

In addition, IHE Radiology is advancing **Manifest-Based Access to DICOM Objects (MADO)**, which supports the structured exchange of a manifest identifying specific DICOM objects—such as relevant prior studies, series, or images—associated with a current clinical encounter. By enabling systems to reference and retrieve imaging objects that are highly likely to be clinically relevant, rather than requiring manual search across entire prior studies, MADO supports more efficient workflows and more reliable data access. Such structured data access is particularly important in AI-enabled environments where relevance, context, and traceability are essential.

Link to MADO Public Comment Draft:

https://www.ihe.net/uploadedFiles/Documents/Radiology/IHE_RAD-Suppl_MADO_Rev1-0_PC_2025-12-08.pdf

Barriers to Innovation and Adoption

From an interoperability and implementation perspective, key barriers include inconsistent use of standards for AI-related data exchange, limited mechanisms for tracing AI outputs to source data and model versions, and the absence of scalable approaches for post-

deployment performance monitoring. These gaps frequently force providers and innovators into bespoke, vendor-specific integrations that do not scale and inhibit competition.

IHE's experience in imaging environments illustrates these challenges clearly. AI outputs must be represented in standardized formats, stored alongside imaging studies, retrieved in predictable ways, and subject to structured review and assessment. The AIR and AIRA Profiles were developed specifically to address these integration and traceability concerns within diagnostic imaging workflows, enabling structured representation of AI findings and documentation of human or system assessment of those results.

Evaluation and Post-Deployment Monitoring

IHE strongly supports HHS's emphasis on real-world evaluation of AI systems. Pre-deployment validation alone cannot ensure sustained safety, equity, and effectiveness across diverse care environments.

Interoperability-enabled evaluation approaches—including standardized representations of AI inputs, outputs, confidence measures, and audit trails—are essential to support ongoing monitoring, bias detection, and performance benchmarking.

The AIRA Profile, for example, defines mechanisms to document structured assessment of AI results, enabling systems to record whether AI outputs were accepted, modified, or rejected, and to support quality monitoring and post-market surveillance activities. This type of standards-based assessment infrastructure can contribute meaningfully to the governance and oversight models contemplated in this RFI.

Federal support for evaluation frameworks that rely on interoperable data exchange, rather than proprietary reporting mechanisms, would materially strengthen the AI ecosystem.

Supporting Accreditation and Industry-Driven Testing

IHE encourages HHS to continue supporting and aligning with private-sector conformance testing, accreditation, and certification activities that validate not only algorithmic claims, but real-world interoperability and workflow integration.

IHE's Connectathon testing program provides a structured, multi-vendor environment where interoperability capabilities—including AI-related integration—can be validated. Vendors that successfully demonstrate interoperability through multi-vendor testing may be recognized through mechanisms such as the **IHE Connectathon SEAL**, which provides a transparent, non-regulatory signal that specific interoperability capabilities have been objectively tested.

ASTP/ONC's long-standing collaboration with IHE demonstrates how public-private partnerships can reduce provider burden while increasing predictability, safety, and trust—outcomes that are directly applicable to AI-enabled clinical systems.

Offering a Practical, Testable Execution Pathway

While ASTP/ONC and many healthcare stakeholders are familiar with IHE's role, other federal partners involved through HHS's interagency coordination may be less aware of existing mechanisms that already support real-world validation and testing of complex health IT capabilities.

IHE respectfully offers its experience and infrastructure as a practical execution pathway that HHS and its federal partners could leverage to explore scalable, non-regulatory approaches to AI validation and oversight.

Specifically, IHE provides:

- A neutral, multi-stakeholder forum where vendors, providers, and regulators collaboratively define implementable requirements aligned with policy goals
- Operational implementation guidance that translates standards and policy intent into deployable system behavior
- Public, real-world testing environments (Connectathons) where interoperability, workflow integration, and traceability can be validated across multiple vendors and systems
- Feedback loops that surface real-world implementation challenges early, before they become systemic risks

Rather than creating new federal testing programs or bespoke validation frameworks, HHS could consider how existing, industry-supported mechanisms—such as IHE Profiles and Connectathons—might be used or extended to evaluate interoperability, auditability, and workflow integration of AI-enabled clinical systems in a practical, repeatable manner.

IHE has recently launched a new program, IHE PRISM, designed to prioritize interoperability in support of key market imperatives. This framework provides an ideal structure to accelerate the adoption and responsible use of artificial intelligence in clinical care. The IHE team looks forward to exploring with ASTP/ONC and other stakeholders how this new approach can help advance shared goals in this area.

IHE does not regulate vendors or assess clinical efficacy. However, decades of experience demonstrate that transparent, collaborative testing of real-world system behavior materially improves safety, reduces implementation burden, and accelerates responsible adoption. These same principles are directly applicable to AI-enabled clinical care.

Looking Forward

IHE views the adoption of AI in clinical care as a continuation of longstanding federal health IT priorities rather than a departure from them. The same principles that enabled nationwide electronic health record adoption, imaging exchange, and health information

networks—clear expectations, implementable guidance, and real-world testing—are equally necessary to ensure AI delivers on its promise.

IHE appreciates ASTP/ONC’s leadership and sustained partnership and stands ready to continue supporting HHS and its federal partners in advancing practical, testable approaches to AI adoption that enhance patient safety, clinical effectiveness, and public trust.

Respectfully submitted,

Integrating the Healthcare Enterprise (IHE)

A handwritten signature in black ink, appearing to read 'A. Goldmuntz', with a stylized flourish at the end.

Aaron Goldmuntz
Managing Director
IHE International
www.ihe.net